

1. Administrative & Study Identification

Full Study Title: Retrospective Cohort Study to Assess Utilization and Effectiveness of Ritlecitinib in a Real-World Population With Severe Alopecia Areata in the United States **Short Title/Acronym:** Utilization and Effectiveness of Ritlecitinib in a Real-World Population With Severe AA in the US **Protocol Number:** NCT07226531 **NCT Number:** NCT07226531 **Posting ID:** 35324603108457459 **Sponsor/Company Name:** Pfizer **Investigational Product:** Ritlecitinib (Litfulo) **Phase of Development:** Phase Not Applicable (Observational / Retrospective Cohort Study) **Trial Status:** Not Yet Recruiting **Disease Area:** Alopecia Areata **Medical Monitor:** Pfizer CT.gov Call Center (ClinicalTrials.gov_Inquiries@pfizer.com / 1-800-718-1021)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate patient characteristics, treatment patterns, and related clinical outcomes of patients who initiated ritlecitinib to treat severe Alopecia Areata (AA) in real-world clinical practice. **Secondary Objectives:** To evaluate the changes in SALT (Severity of Alopecia Tool) score and landmark probabilities of achieving clinically meaningful scalp hair regrowth (e.g., SALT $\leq 20\%$ and SALT $\leq 10\%$) at 6-, 12-, 18-, and 24-months post-index date.

2.2 Background & Rationale To address the gap in understanding the clinical effectiveness of ritlecitinib in real-world clinical practice. Ritlecitinib, a novel JAK3/TEC family kinase inhibitor, was FDA-approved in June 2023 for severe AA in adults and adolescents aged 12 years or older. While robust information exists from clinical trials (like the ALLEGRO studies) establishing its safety and efficacy, less is known about its utilization, effectiveness, and treatment patterns (persistence, discontinuation) in everyday clinical settings. This study generates real-world evidence to better inform guidelines and improve treatment access for this patient population.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Retrospective Cohort Study) **Control Type:** Single-arm (Real-world cohort) **Blinding:** Open-label (Retrospective Medical Record Abstraction) **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$\$\$: 300 patients **Number of Sites:** Real-world clinical practices (Dermatology sites) in the United States **Estimated Study Start:** Late 2025 **Estimated Primary Completion:** 31-Mar-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Confirmed diagnosis of severe AA (as determined by the treating physician). 2. Initiated ritlecitinib for severe AA between 01 July 2023 and 28 February 2025. 3. At least 12 years of age at ritlecitinib initiation. 4. Has at least one additional follow-up visit/consultation after ritlecitinib initiation in which scalp hair loss was assessed. 5. Has a complete medical record regarding management or treatment of AA, covering the period of initial AA diagnosis until their most recent follow-up/consultation.

4.2 Exclusion Criteria 1. Ever diagnosed with other types of alopecia (i.e., e.g., chemotherapy-induced alopecia, androgenic alopecia [i.e., male pattern baldness], cicatricial alopecia [i.e., scarring alopecia], postpartum alopecia, or traction alopecia), other diseases (besides AA) that can cause hair loss, or other diseases that could interfere with assessment of hair loss/regrowth. 2. No documentation in the medical record of percent of scalp hair loss. 3. Participated in any clinical trial involving treatment with ritlecitinib, baricitinib, deuruxolitinib, delgocitinib, brepocitinib, or upadacitinib.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Ritlecitinib oral capsules (e.g., 50 mg) once daily as prescribed per label in real-world practice. **Route of Administration:** Oral **Duration of Treatment:** Initiated at least 6 months prior to data collection, with continuous real-world use monitored up to 24 months post-index.

5.2 Concomitant & Prohibited Therapy Permitted: Concomitant therapies as prescribed by the treating physician in a real-world setting. **Prohibited:** Not recommended for use in combination with other JAK inhibitors, biologic immunomodulators, cyclosporine, or other potent immunosuppressants.

6. Study Timeline & Visit Schedule

(Note: As a retrospective observational study, visits align with standard clinical practice)

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening / Baseline	Index Date Chart

review: Confirm AA diagnosis, eligibility, extract baseline SALT score, and index date of ritlecitinib initiation | | **Interim** | Months 6, 12, 18 | Chart review: Extract follow-up SALT scores, treatment adherence, and AE data documented in EMR | | **End of Study** | Month 24 | Final data extraction for long-term effectiveness, treatment persistence, and landmark SALT probability |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: 1. Patient demographics characteristics: race (Number of percentages of patients by race). 2. Patient demographics characteristics: sex, age at initial AA diagnosis, geographic location of patient's treating dermatologist, insurance type (Number of percentages of patients by sex). 3. Patient demographics characteristics: age at initial AA diagnosis (Number of percentages of patients by age at initial AA diagnosis). *(Additionally evaluating SALT score overall change from baseline and landmark probabilities of achieving SALT ≤ 20 and SALT ≤ 10 at 6-, 12-, 18-, and 24-months post-index date).* **Measurement Method:** Retrospective Electronic Medical Record (EMR) / Chart Abstraction.

7.2 Secondary & Exploratory Endpoints Patient real-world treatment patterns, treatment persistence, and discontinuation rates (including reasons for discontinuation such as insufficient response).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Retrospective extraction of safety events, tolerability issues, and reasons for discontinuation documented by the treating physician in the EMR. **Serious Adverse Events (SAEs):** Extracted from available medical records. **Data Monitoring Committee (DMC):** N/A (Retrospective observational study).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: All eligible patients in the real-world cohort who initiated ritlecitinib for severe AA. **Sample Size Justification:** Descriptive statistics on a target cohort of approximately 300 patients to accurately detect and capture meaningful real-world utilization trends and effectiveness outcomes. **Handling of Missing Data:** Standard imputation for missing EMR data or complete case analysis as applicable.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: The study data collection will be conducted in accordance with Good Pharmacoepidemiology Practices (GPP) and patient data privacy regulations. **Monitoring Plan:** Source data verification of abstracted records and EMR audits for accuracy. **Audit Trail:** Secured electronic data capture (EDC) system for abstracted clinical data.

11. Study Identifiers & Governance

Primary ID: NCT07226531 **Registry Number:** NCT07226531 **Posting ID:** 35324603108457459 **Sponsor/Lead Organization:** Pfizer **Study Title:** Utilization and Effectiveness of Ritlecitinib in a Real-World Population With Severe AA in the US **Official Regulatory Title:** Retrospective Cohort Study to Assess Utilization and Effectiveness of Ritlecitinib in a Real-World Population With Severe Alopecia Areata in the United States

12. Clinical Framework (Alopecia Areata Specific)

Primary Condition: Severe Alopecia Areata (AA) **Diagnosis Threshold:** Severe scalp hair loss (e.g., SALT score $\geq 50\%$) **Medical Methodology:** JAK3/TEC family kinase inhibitor therapy (Ritlecitinib) **Optimization Target:** Clinically meaningful scalp hair regrowth (Target: SALT $\leq 20\%$)

13. Study Procedures & Methodology

Management Pathway: Real-world dermatology clinical practice management **Education Protocol (HCP):** N/A (Observational study; treatment per label) **Education Protocol (Patient):** N/A (Standard of care) **Quality Audit Mechanism:** Medical record abstraction review and EMR data validation

14. Observation & Follow-Up Schedule

Total Duration: Retrospective follow-up of up to 24 months post-index **Onsite Visit Cadence:** Retrospective EMR data abstraction at Baseline, 6, 12, 18, and 24 months **Remote Monitoring Frequency:** N/A (Retrospective chart review) **Checklist Review Interval:** Routine clinical follow-up EMR extraction points

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	02-Apr-2026				

Clinical Operations Lead | [Name] | ____ | 02-Apr-2026 | | Lead
Statistician | [Name] | _____ | 02-Apr-2026 |